variant was also found in samples of folks testing positive in northern parts of the country: Delhi, Punjab and Haryana. What seemed to have compounded the problem was that since daily new cases were at their lowest points in India, people had let their guards down. COVID-19 protocol wasn't being strictly followed or enforced by governments. Relaxations had been given to resume travel, activity and social events. It was perhaps this intermingling of undetected COVID-19-infected persons —some of whom were attributed to the original strain, while some were due to Alpha—which provided a fertile host for the pathogen to proliferate and evolve into a more fatal variant like the Delta.

By March, the Maharashtra government had managed to bring down the positivity rate in the high-impact districts to about 12 per cent. However, a new disturbing trend of the virus spreading to semi-rural areas began to appear. Just like in Maharashtra, transmissible variants were also spreading rapidly in other parts of the state and country. Since most cases were asymptomatic, the initial spread remained largely undetected. To give you another example, let us take Delhi's case. The Alpha variant was so quick in its propagation that in January it had just begun to get detected in the genome sequencing samples in Delhi. By February, about 20 per cent of all positive samples in the Delhi NCR region were attributable to it and by March that percentage had increased to 40 per cent. Parallel to this, let us look at the speed of the Delta spread. Towards the end of February, 5 per cent of samples from Delhi were due to the Delta variant, in March that percentage had increased to 10 per cent. By April, when one in every three people were testing positive for COVID-19, over 60 per cent of positive cases in the region were caused by the Delta variant. A team of scientists from the Institute of Genomics and Integrative Biology that did extensive studies on the Alpha and Delta variants observed, 'While much more remains to be done, three takeaways for now are: Delta (B.1.617.2) is more transmissible than Alpha (B.1.1.7), there seems to be greater immune escape and reinfection, and fully vaccinated breakthroughs were disproportionately due to Delta.'[5]

All signs pointed towards a possibility of the next wave upon us. This wasn't a surprise. In September 2020, the Dr Paul-led empowered group had presented its report to the PMO. It looked at how India would need to prepare to deal with this increased caseload now that a second wave was inevitable and that India would likely see peaks of about three lakh cases a